



# Epistaxis

## History

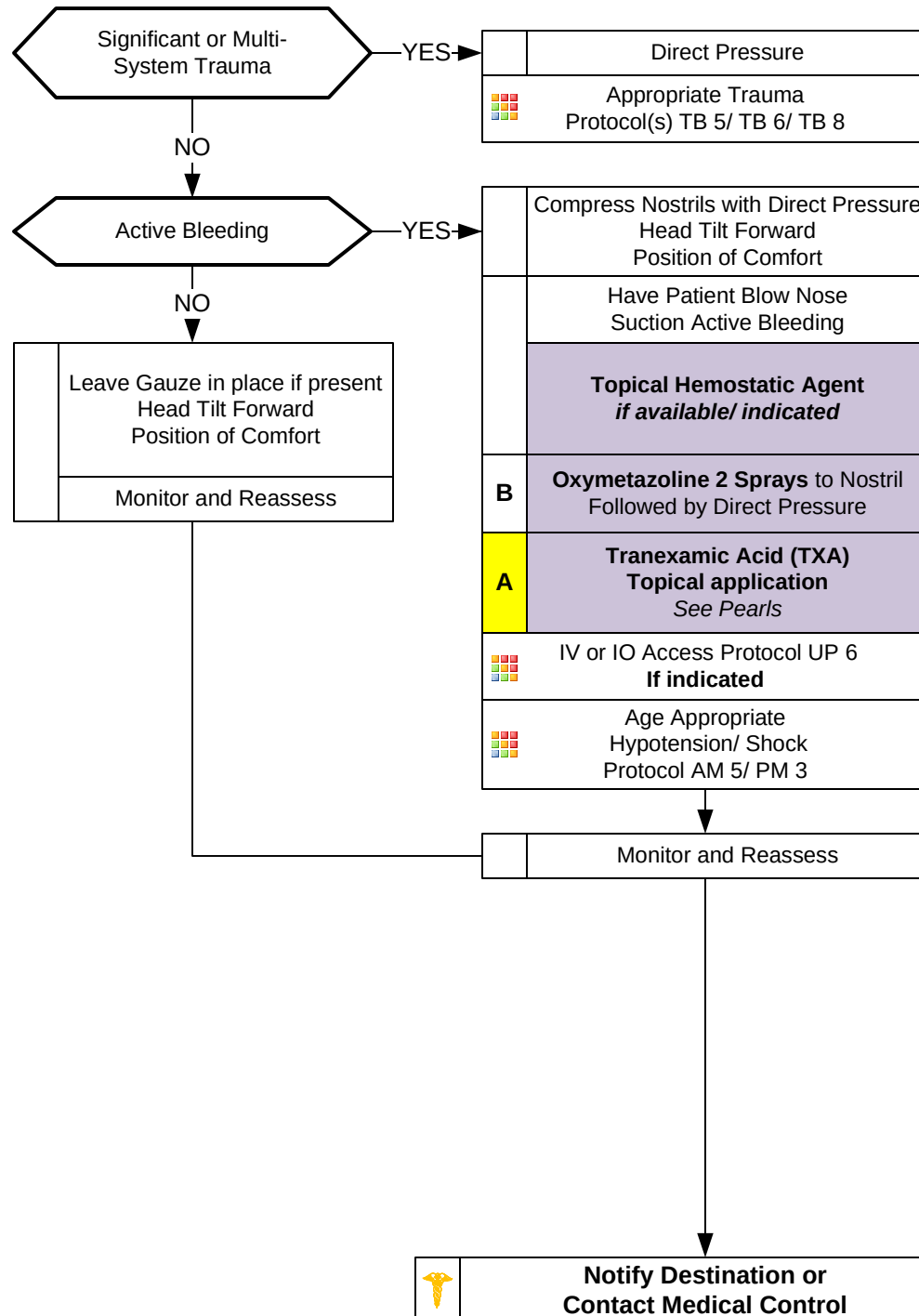
- Age
- Past medical history
- Medications (HTN, anticoagulants, aspirin, NSAIDs)
- Previous episodes of epistaxis
- Trauma
- Duration of bleeding
- Quantity of bleeding

## Signs and Symptoms

- Bleeding from nasal passage
- Pain
- Nausea
- Vomiting

## Differential

- Trauma
- Infection (viral URI or Sinusitis)
- Allergic rhinitis
- Lesions (polyps, ulcers)
- Hypertension



### Age Specific Blood Pressure indicating possible shock

Age 0 – 28 days: SBP < 60  
Ages ≥ 1 month: SBP < 70  
Age 1 – 9: SBP < 70 + (2x Age)

Ages 10 – 64: SBP < 90  
Ages ≥ 65: SBP < 110

All ages Shock Index:  
HR > SBP



# Epistaxis

## Pearls

### **Procedure:**

- Apply 10 ml of a 1 g/10 ml Tranexamic acid (TXA) solution to a gauze pad
- Insert tranexamic acid-soaked gauze into nasal cavity
- Gauze may be removed after 20 minutes, and bleeding has stopped.

### • **Recommended Exam: Mental Status, HEENT, Heart, Lungs, Neuro**

#### • **Tranexamic acid (TXA) Use in Epistaxis:**

May be used topically if approved by agency Medical Director.

Procedure should be created with specific guidance on how to administer Tranexamic acid (TXA) for epistaxis.

No clear evidence that Tranexamic acid (TXA) provides benefit over conventional vasoconstrictors and sustained direct pressure.

- It is very difficult to quantify the amount of blood loss with epistaxis.
- Bleeding may also be occurring posteriorly. Evaluate for posterior blood loss by examining the posterior pharynx.
- Anticoagulants include warfarin (Coumadin), Apixaban (Eliquis), heparin, enoxaparin (Lovenox), dabigatran (Pradaxa), rivaroxaban (Xarelto), and many over the counter headache relief powders.
- Anti-platelet agents like aspirin, clopidogrel (Plavix), aspirin/ dipyridamole (Aggrenox), and ticlopidine (Ticlid) can contribute to bleeding.